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SciScore Report

Below you will find your SciScore report containing three tables. Your score is calculated based on adherence to scientific rigor criteria (Table 1) and identification of key biological resources (Table 2). Table 3 contains statistical tests and oligonucleotides but is not scored. If SciScore makes any mistakes, please contact us to help us learn and improve.

Table 1: Rigor Adherence Table

Ethics
IRB: 7 Standard protocol approvals and patient consents All participating centers obtained approval from their corresponding ethics committees to participate in the NMD-ES Registry, and all patients signed an informed consent form for use of their data for scientific purposes.
Consent: 7 Standard protocol approvals and patient consents All participating centers obtained approval from their corresponding ethics committees to participate in the NMD-ES Registry, and all patients signed an informed consent form for use of their data for scientific purposes.
Inclusion and Exclusion Criteria
We excluded patients lost to follow-up and patients for whom relevant information was missing.
Attrition
not detected.
Sex as a biological variable
not detected.
Subject Demographics
Age: not detected.
Weight: not detected.
Randomization
not detected.
Blinding
not detected.
Power Analysis
not detected.
Replication

not required.

Data Information

Availability: Data availability statement The data that support the findings of this study are available upon request from the corresponding author.

Table 2: Key Resources Table

Your Sentences	REAGENT or RESOURCE	SOURCE	IDENTIFIER
Antibodies			
The demographic and clinical variables analyzed were: sex, age at onset, AChR and MuSK antibody positivity, severity and distribution of muscle weakness according to the Myasthenia Gravis Foundation of America (MGFA) clinical classification 12 at onset and at maximal worsening, frequency of life-threatening events (defined as MGFA IVB and V) and myasthenic crises (MGFA V) at onset and at maximal worsening, generalization--defined as patients with a focal ocular form of the disease at onset (MGFA I) but later generalized (MGFA II or higher), thymoma defined by pathologic study in patients undergoing thymectomy and by thoracic CT in the others, thymectomy, follow-up time defined as the difference between the follow-up deadline or date of death and the date of diagnosis, and clinical outcome according to the MGFA post-intervention status (MGFA-PIS) 12 at the end of follow-up.	MuSK		
MG standard of care in Spain The current Standard of care to treat MG in Spain includes: (1) acetylcholinesterase inhibitors as symptomatic therapy (2) immunosuppressive drugs, being steroids the first-line immunosuppressive treatment, azathioprine and mycophenolate mofetil as second line immunosuppressors or steroid-sparing agents, and others (cyclosporine, tacrolimus, rituximab, methotrexate, and cyclophosphamide) in case of side effects or lack of response (3) thymectomy, performed in patients with thymoma and also in patients younger than 65 years with anti-AChR antibodies and no thymoma and (4) immunomodulatory therapies (intravenous immunoglobulin and plasma exchange), predominantly used in acute worsening or MG crisis.	anti-AChR		

Software and Algorithms			
Data analysis was carried out using the STATA 13 software for Windows (StataCorp LP, College Station, TX).	STATA		Suggestion: Stata (RRID:SCR_012763)(link)
	StataCorp		Suggestion: Stata, RRID:SCR_012763(link)

Other Entities Detected

Your Sentences	Recognized Entity
Statistical Tests	
Differences in baseline characteristics between patient subgroups were evaluated using the chi-squared test to compare categorical variables, and Students t test to compare quantitative variables.	chi-squared test
	t test

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For a full description of scored criteria and tips for improving your score, please see https://www.scicrunch.com/sciscorereport-faq